

Axillary Nerve

Course and Effects of Lesion

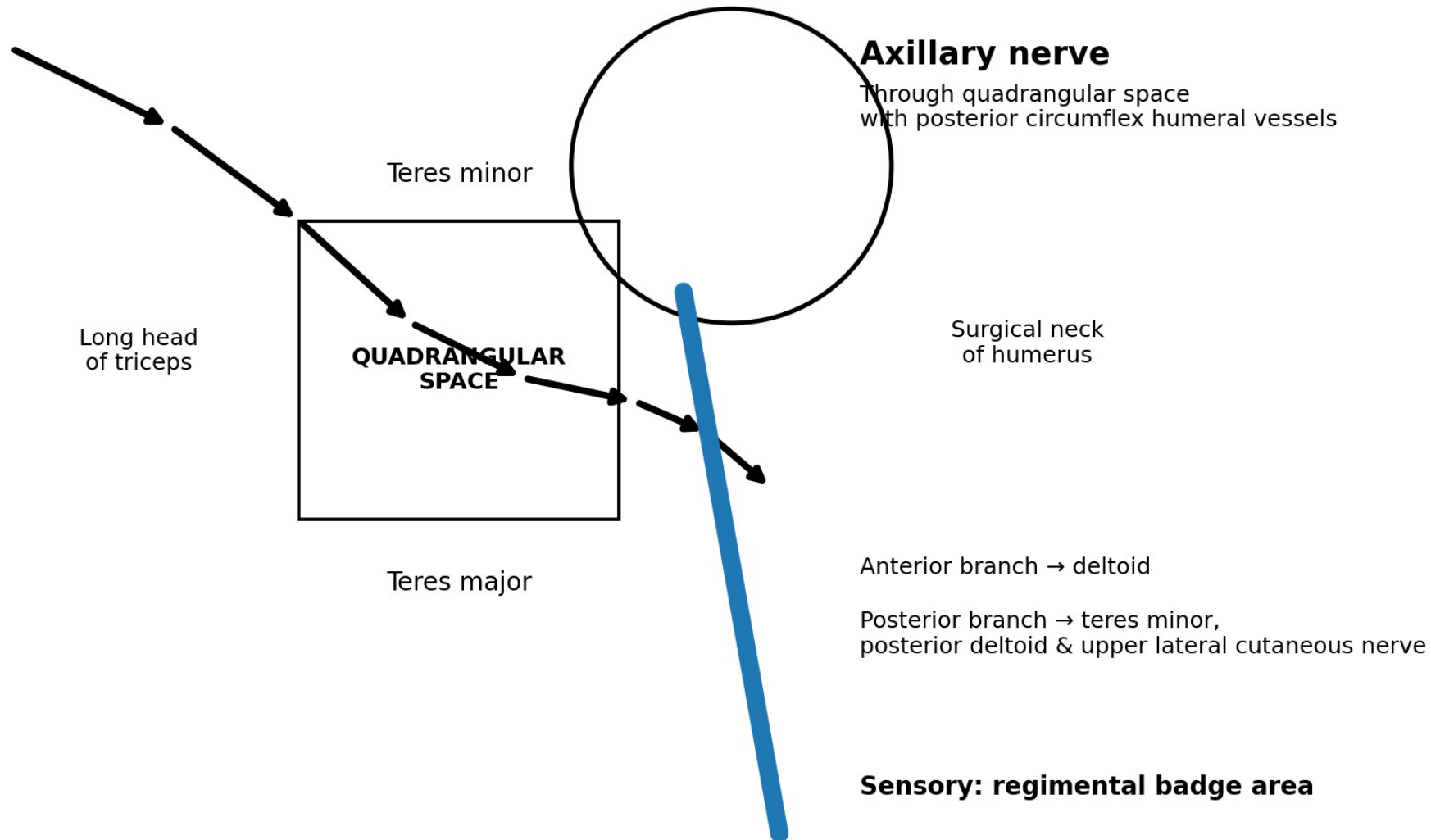
Root values: C5–C6.

Motor: deltoid and teres minor.

Sensory: skin over lateral shoulder (regimental badge area).

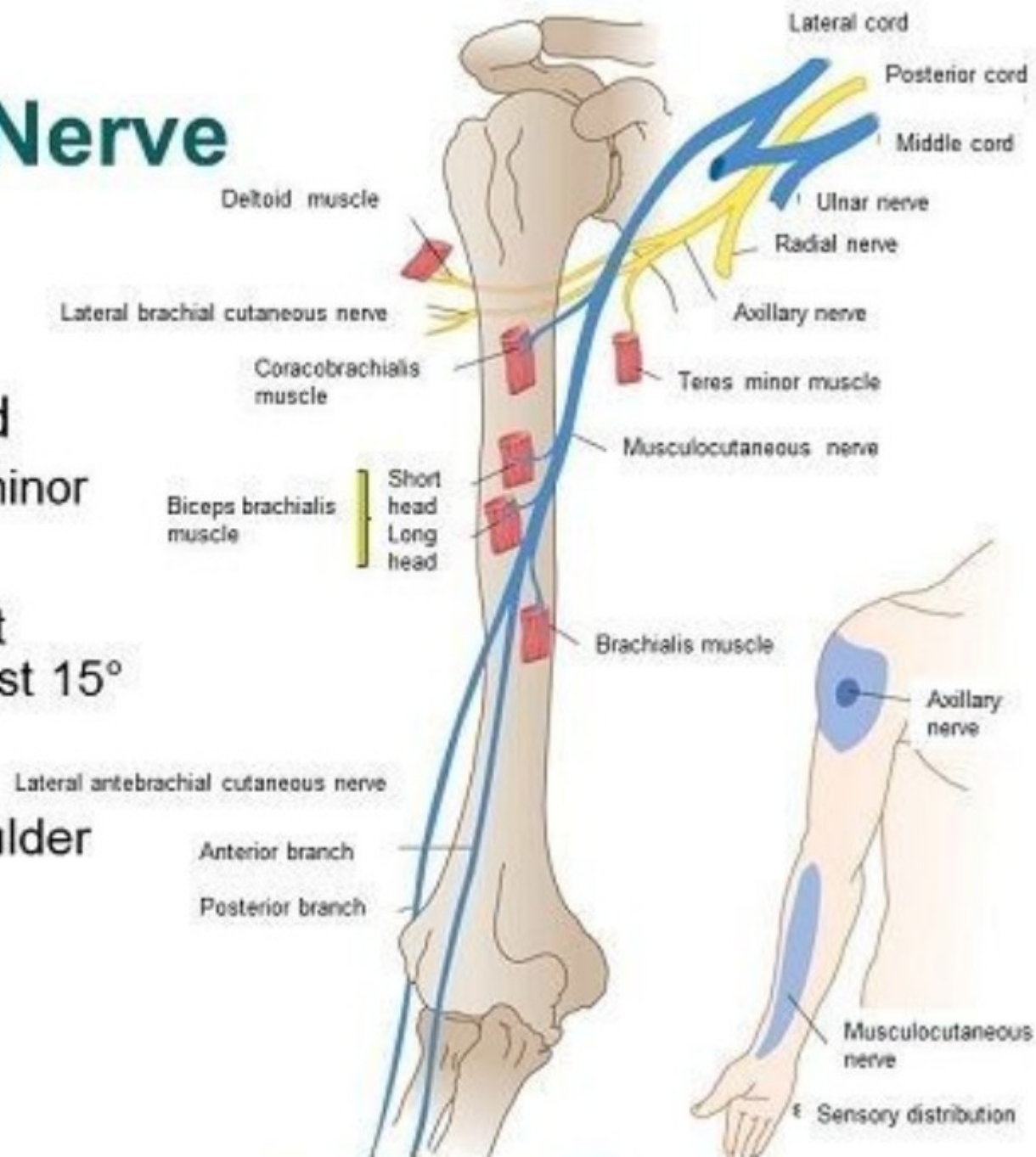
Course of the Axillary Nerve

Posterior cord
C5-C6



The Axillary Nerve

- Muscles innervated
 - Deltoid and teres minor
- Motor functions
 - Abduction of arm at shoulder beyond first 15°
- Sensory
 - Skin over the shoulder



Effects of Axillary Nerve Lesion

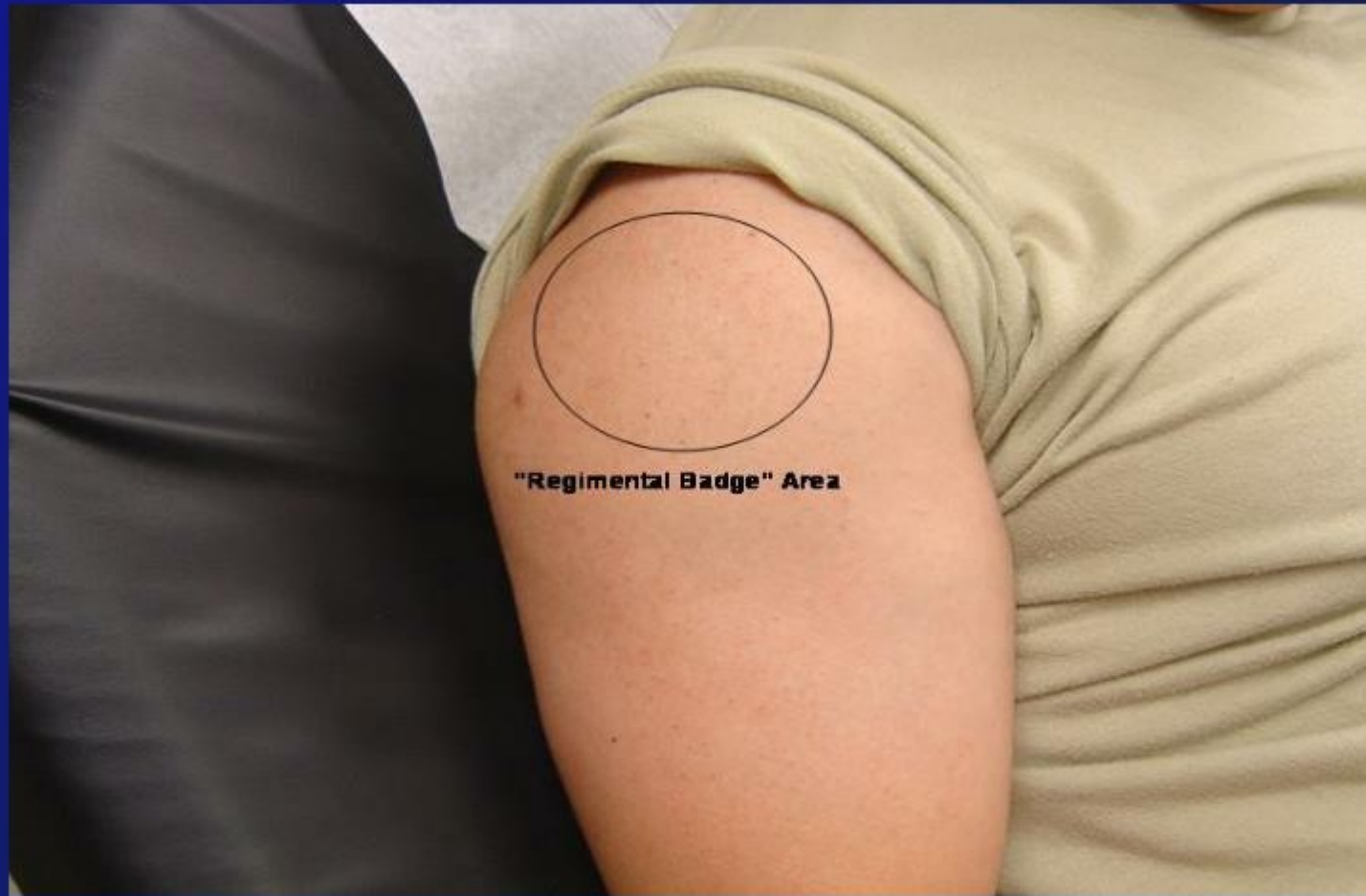
Deltoid weakness → impaired shoulder abduction beyond the initial 15°.

Teres minor weakness → reduced external rotation may occur.

Sensory loss: numbness/paraesthesia over lateral shoulder ('regimental badge' area).

Long-standing lesion → deltoid wasting and flattened shoulder contour.

Common causes: anterior shoulder dislocation, surgical-neck humerus fracture, traction or surgical injury.



The "regimental badge" area → Examine **pin-prick** sensation to this area to **assess** axillary nerve **sensory** function.

Clinical Examination

Test deltoid power by resisted shoulder abduction.

Assess sensation over the lateral deltoid / regimental badge area.

Look for deltoid atrophy or loss of shoulder contour.

After shoulder trauma or dislocation, document axillary nerve function before and after reduction.

Exam Summary

C5–C6 → posterior cord → quadrangular space → around surgical neck of humerus.

Motor: deltoid + teres minor.

Sensory: regimental badge area.

Lesion classically causes weak shoulder abduction + lateral shoulder sensory loss.

Commonly injured in anterior shoulder dislocation and proximal humerus surgical-neck fractures.